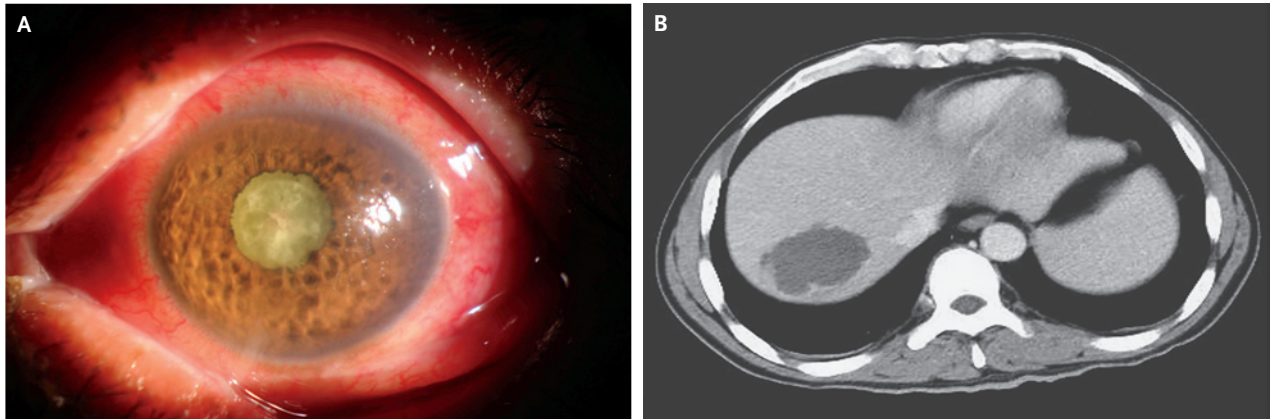


IMAGES IN CLINICAL MEDICINE

Chana A. Sacks, M.D., *Editor*Hypervirulent *Klebsiella pneumoniae*

A 45-YEAR-OLD MAN WITH TYPE 2 DIABETES MELLITUS PRESENTED TO THE ophthalmology clinic with a 5-day history of pain and decreased vision in his left eye. His temperature was 39.2°C, and his pulse was 144 beats per minute. Examination of the left eye showed diffuse conjunctival injection, a dense cataract, and no light perception (Panel A). Owing to concern for endogenous endophthalmitis with systemic infection, the patient was referred to the emergency department. Contrast-enhanced computed tomography of the chest, abdomen, and pelvis was performed and showed abscesses in the lungs, liver (Panel B), kidneys, and prostate. The patient received intravenous and intravitreal antibiotic agents and underwent percutaneous drainage of the abscesses in the liver and prostate. Cultures of blood, pus from the abscesses in the liver and prostate, and vitreous fluid grew *Klebsiella pneumoniae*, and the hypervirulent K1 serotype was identified on a polymerase-chain-reaction assay. Hypervirulent *K. pneumoniae* can cause pyogenic abscesses in a range of sites, including the liver, central nervous system, lungs, urinary tract, and soft tissues. The patient declined vitrectomy during hospitalization. At a follow-up visit 2 months after presentation, he had recovered well but had complete loss of vision in his left eye.

DOI: 10.1056/NEJMicm2101602

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Qiong Zou, M.D.

Affiliated Eye Hospital of Nanchang
University
Nanchang, China

Yang Li, M.D.

First Affiliated Hospital of Nanchang
University
Nanchang, China
liyang092792@126.comThis article was published on August 21,
2021, at NEJM.org.